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Democracy Dies in Darkness

STORYLINE

End-of-life care: An industry with soaring profits, funded by taxpayers



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As Peter Whoriskey [reports today](#), the American hospice industry is booming. It can offer Americans a new source of care as they approach their last days.

But what happens when hospices, in part to improve profits, attempt to care for people who *aren't* terminally ill? Whoriskey wrote about a 77-year-old North Carolina man, Clinard “Bud” Coffey, who entered hospice care for pain management — and died two weeks later:

“His death certificate, which was signed by the hospice doctor, listed the cause as ‘renal cell carcinoma’ or kidney cancer. But that doctor had never examined Coffey, his family said, and medical records from just a few weeks earlier do not mention it.

‘My dad wasn’t dying of cancer,’ said his son, Jeff Coffey. ‘Once he was on hospice, their answer for everything was more drugs. Everything we know about his death is consistent with an overdose.’”

Before you consider hospice care, know the facts:

1. More than a million patients every year receive hospice care in the United States.

About [66 percent of hospice patients](#) spend their final days at home. Many receive just weekly nursing visits. In 2011, more than 5,300 hospice operations in the United States accepted Medicare, according to the National Hospice and Palliative Care Organization.

2. To [qualify for hospice care](#) under Medicare, a doctor and a hospice medical director must agree that a patient has six months (or less) to live.

Patients must also forsake “curative” treatments, according to Kaiser Health News.

3. End-of-life care is a \$17 billion industry.

Hospice care is now dominated by for-profit companies, Whoriskey reports. It’s largely supported by the U.S. government; Medicare funded roughly \$15 billion of industry revenue last year.

Some companies pay \$100-a-head for patient referrals, according to Bloomberg.

4. Patients are staying in hospice care longer — and profits are soaring.

California offers some insight into the industry as a whole. The average length of a stay in California hospice care has increased over the past decade: Profit per patient quintupled to \$1,975, records show.

5. The number of ‘hospice survivors’ has recently surged.

Companies pad profits by recruiting patients who aren’t terminally ill, Whoriskey reports. And healthier patients require fewer visits while staying enrolled longer. The proportion of patients who were discharged alive jumped about 50 percent between 2002 and 2012, according to a Post analysis.

6. Not all patients receive what doctors consider adequate medical attention during emergencies.

Pain, breathing problems and seizures, for example, sometimes flare up in ways that cannot be controlled without special attention. For those cases, hospices should offer either “continuous” nursing care at home or inpatient care at a medical facility, Whoriskey reports. But about one in six American hospice outfits — serving more than 50,000 of the terminally ill — did not administer crisis care to any patients in 2012, according to an analysis of millions of Medicare billing records.

